USER INFORMATION

INFUSOL HMK
DARROW'S SOLUTION INTRAVENOUS INFUSION BP

Full electrolyte solution for replacement of extracellular fluid.

Composition:
The Solution contains

Sodium Chloride BP	4.00 g/L
Potassium Chloride BP	2.60 g/L
Sodium Lactate USP	5.94 g/L

Electrolytes:	**mmol/L**
Sodium Ion (Na^+)	121.0
Potassium Ion (K^+)	35.0
Chloride Ion (Cl^-)	103.0
Lactate Ion (HCO_3^-)	53.0

Osmolarity : 312 mOsm/L

Characteristics:
Darrow's Solution Intravenous Infusion has a similar electrolyte composition as the extracellular fluid. The lactate ion is gradually metabolised and converted to bicarbonate in a concentration also resembling that of extracellular fluid. Additionally the lactate ion exerts a slightly alkalinising effect.

Indication:
Replacement of extracellular fluid loss (isotonic dehydration).
Sodium depletion, light metabolic acidosis.

Dosage:
Average Dose : 2000mL/day
Drop rate : 120-180 drops/min corresponding to 360 – 540 mL/h.

Route of administration : Intravenous

Contraindications:
-Hypertonic and hypotonic dehydration.
-Hyperhydration, Oedema
-Alkalosis
-Hypokalemia, hypernatraemia, hyperlactatemia.
-Renal insufficiency
-Hypertension

Precaution :
In case of liver disease where lactate metabolism is severely impaired or in anoxic shock or cardiac failure. In cardiac failure, lactic acidosis takes place which is more in heart tissues. The lactate given cannot be utilised by the tissues and therefore it worsens the condition.

Treatment of overdosage/Adverse effects:
Symptoms : Excessive or too rapid administration may produce alkalosis. Severe alkalosis may be accompained by hyperirritability or tetany.
Treatment : Discontinue compound sodium lactate (Hartmann's Solution). Control symptoms of alkalosis by rebreathing expired air from a paper bag or rebreathing mask or, if more severe, by parenteral injection of calcium gluconate. Correct severe alkalosis by IV infusion of 2.1% ammonium chloride solution, except in patients with hepatic disease, in whom ammonium use is contraindicated. Sodium chloride (0.9%) IV or potassium chloride may be indicated if there is hypokalemia. Administer calcium gluconate to control tetany.

Caution:
Not for use in the treatment of lactic acidosis.
The compatibility of any additives to this solution should be checked before use.
Single dose container. Discard all unused contents. Do not use if leakage is detected.
If any visible solid particles, growth or turbidity appears during storage, the product should not be used.

Storage:
Do not store above 30°C.

Expiry Date:
3 years from manufacturing date.
Do not use after expiry.

Presentation:
500mL in medical plastic collapsible bag.
500mL PE Plastic bottle.

Manufactured by:

AIN MEDICARE SDN. BHD.
Jalan 6/44,
Kawasan Perindustrian Pengkalan Chepa 2,
16100 Kota Bharu, Kelantan,
MALAYSIA.

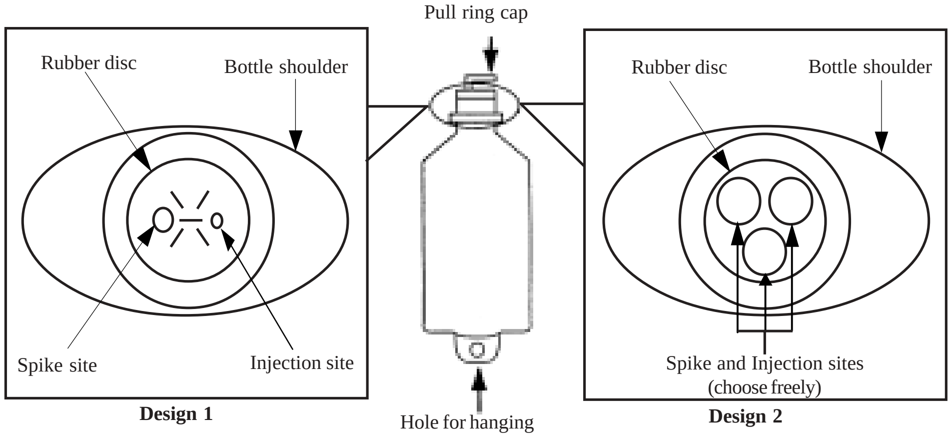
Date of Revision: 11.04.05

HANDLING INSTRUCTION

AIN MEDICARE INTRAVENOUS INFUSION FLUID IN POLYETHYLENE(PE) BOTTLE

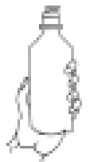
The bottle is made from injectable grade polyethylene.

PARTS OF THE PLASTIC BOTTLE



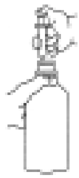
Top view after pull ring cap is removed. Please refer whichever applicable design.

INSTRUCTION FOR THE USAGE OF IV ADMINISTRATION SET & ADDITION OF DRUG

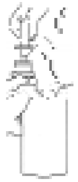


1. Before use check for any solid particle, growth, turbidity or leakage. Discard the product if particle, growth, turbid or leakage is found.

2. Pull the cap's ring until completely removed.



3. Additives may be injected through **injection site** (Refer to Design 1 or 2 whichever applicable) on the rubber disc. Mix the solution thoroughly.



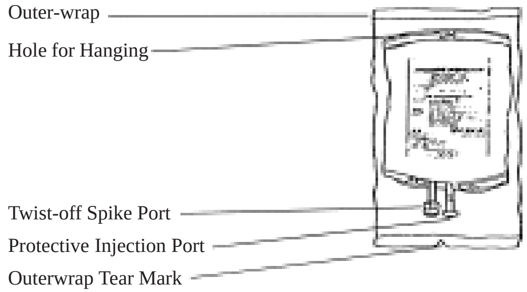
4. Hold bottle down firmly and insert the spike of administration set into the **spike site**. (Refer to Design 1 or 2 whichever applicable).

(Spike site is meant for spiking of the administration set and is only for single use)

AIN MEDICARE INTRAVENOUS INFUSION FLUID IN PVC MEDICAL COLLAPSIBLE BAG

The Intravenous solution in medibag is packed and sterillized in an outerwrap.

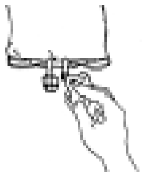
PARTS OF THE COLLAPSIBLE BAG



INSTRUCTION FOR THE USAGE OF IV ADMINISTRATION SET & ADDITION OF DRUG

1. Remove outerwrap by tearing from the tear mark when ready to use. The outerwrap is a moisture barrier and the inner bag (medibag) maintains sterility of the product.

2. After removing the outerwrap check for any solid particle, turbidity or growth. Squeeze the medibag firmly to test for leakage. Discard the product if particle, turbidity, growth or leakage found.



3. Additives may be injected through injection port. Remove the aluminium cap. Disinfect the injection port.

4. Hold the injection port and inject through the rubber piece. Mix thoroughly with the solution.

5. Twist-off the tab to expose the spike-port. Insert spike of the I.V. administration set into the spike port aseptically.